

Expert Interview Summary — Dr Philip Choy

EngD Research Record — Expert Consultation Series (fourth consultation)

Project: Leveraging Gamification as a Proactive Approach for Continuous Improvement and Innovation Enhancement

Researcher: Sidney K. T. Tam · Lead Supervisor: Prof. Benny C. F. Cheung

DRAFT for author review — prepared 4 August 2026

1. Participant and standpoint

Dr Philip Choy is a certified naturopathic physician (ANMCB Cert. 06394, 2008) and Doctor of Natural Medicine (ACN, 2008), practising in Hong Kong for over twenty-five years in clinical naturopathic and complementary medicine. His roles include President of the Hong Kong Academy of Naturopath (2010), ANMCB Examiner for Hong Kong and Macau, Certified Clinical Dietitian (NHI-PRC, 2007), Osteopath (OCO, Canada, 2015), and faculty appointments at the Osteopathic College of Ontario and the Da Vinci Institute of Holistic Medicine. He reports clinical experience with over one thousand elderly patients, predominantly in chronic-disease management.

Standpoint note (positioning). Dr Choy practises naturopathic and complementary medicine; he is not a registered medical practitioner under the Hong Kong Medical Registration Ordinance, and his doctoral title derives from his Doctor of Natural Medicine qualification. His contribution to the panel is therefore positioned as sustained practice-based insight into elderly health behaviour, engagement and adherence, complementing the panel's academic gerontology (Dr Sarah C. W. Yuan), hospital neurology (Dr Gary K. K. Lau) and physiotherapy (Mr Thomas Wong) standpoints. This description should be used verbatim in the thesis so the expertise claim is exact.

2. Consent and records

A signed consent form was received on 4 August 2026: all nine consent statements initialled; participant signature dated 4 August 2026; project title, researcher and supervisor correctly stated. Outstanding on the form: (a) the identification preference shows NEITHER box selected — until Dr Choy ticks one, named attribution is NOT authorised and this summary must be treated as attributable only in anonymised form; (b) the researcher countersignature line is blank; (c) the footer carries the "Draft prepared for ethics review" banner, as on the other panel forms, to be regularised with the HSESC characterisation. The signed form and Dr Choy's 2022 biodata are archived in the thesis evidence folder.

3. Interview logistics — TO BE CONFIRMED BY AUTHOR

Date of interview: [TO CONFIRM — consent is dated 4 Aug 2026; if the conversation took place earlier, record both dates under the dual-dating rule]. Mode: [in person / phone / video — TO CONFIRM]. Duration: [TO CONFIRM]. Language: [TO CONFIRM]. Primary record: researcher's notes [audio recording yes/no — TO CONFIRM; statement 6 permits audio].

4. Summary of Dr Choy's views (researcher's record)

4.1 Practice background. Dr Choy has over twenty-five years of clinical practice in Hong Kong and has treated more than one thousand elderly patients, with a concentration in chronic disease. His observations below draw on that caseload.

4.2 Convergence on the target demographic. In his experience, adults from roughly 65–70 upward who are well educated and financially independent (requiring no government subsidy or social-welfare support) are the segment that engages with advanced health technology — and they do so only once they have identified a concrete benefit, continuous glucose monitoring for pre-diabetic patients being a typical example. He regarded this as the same demographic this research addresses, corroborating the study's target-group definition.

4.3 Engagement drivers. Short-term, visible benefit is critical to this group's participation. A health technology that takes a long time to show improvement risks losing them. Embedding games or similar approaches may help users stay with the technology, but he was explicit that this is conditional — "questionable, depending on the game design."

4.4 Gamification as a retention pathway. Where a health technology cannot quickly show results, a gamified layer can be an alternative means of sustaining a positive mentality toward continued health improvement. The game's rewarding is, in his view, the critical component. He found the idea of AI-designed zero-failure games a very interesting approach, while both parties noted this lies beyond the present research's empirical scope — it is a critical input in support of the research hypotheses, not a tested claim.

4.5 Failure-driven dropout and abandonment. Asked about the pattern in which elderly users drop out after failures and eventually abandon both the game and the health technology, Dr Choy confirmed that if the game or technology continues to deliver failure results and a negative impression, this syndrome is very possible in his experience; he returned to rewards as the critical protective element.

4.6 Attribution of the failure that matters. The researcher put the definitional question directly: is the operative failure the game failure, or the health result the technology delivers? Dr Choy agreed the game itself does not cause the abandonment syndrome (FIAS); it arises mainly from users' concerns and worries about their own health performance — a pattern he described as very common among elderly people who have acknowledged a principal health dysfunction. This is an independent, practice-based statement of the attribution claim at the centre of this thesis: the verdict, not the game, drives abandonment.

4.7 Reward design for this demographic. Because the target group is educated and financially comfortable, simple tokens such as game coins or coupons do not motivate them. The reward system must be more advanced to draw and hold their attention and engagement.

5. Relation to the thesis

(a) FIAS attribution (§5.6, §8.5): point 4.6 is direct practice-based corroboration that failure-induced abandonment originates in the health verdict rather than in game failure — the

proposition Stage 3's frame-sensitivity design tested. (b) Zero-failure design (§6.2): point 4.4 independently endorses the Stage-3 build's zero-failure mode as a design direction. (c) Adaptive-learning framework (§7.5): points 4.3 and 4.7 refine the framework's reward element — short-horizon visible benefit, and reward sophistication matched to an educated, financially independent segment for whom token economies fail. (d) Target-population definition (§6.1): point 4.2 corroborates the educated-cohort framing from an independent clinical caseload. As with the whole panel, these views are interpretation and practice validation of the findings; they are not additional empirical evidence and do not substitute for sample size.

6. Records checklist

Archived: signed consent form (4 Aug 2026); biodata (2022). To file: researcher's interview notes; identification-preference selection; researcher countersignature; interview date/mode/duration/language confirmation for the H.6 consultation register.